



Webinar - Episode 5 (part 2)

Complex Mental Health Claims, scanning documentation for evidence of mental ill-health and letters of instruction

*Presentations by **Dr Lorenzo Grespi** and **Dr William Badenhorst**
(Consultant Psychiatrists and Medical Directors at Grespi) on mental health claims
management in the age of COVID-19, with questions to follow*

22nd October 2020 11:00 to 12:15

The recording of the session is available at this link:

<https://vimeo.com/472128035/b3e408791b>

Health Claims Forum: Grespi webinar 2



Mental health claims:
scanning medical documentation,
instruction letter and complex claims

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Scanning medical documentation – 1



A) If assessment/treatment reports are available, look for:

- formulation
- diagnosis
- prognosis
- treatment outcome: rationale and clear clinical evidence
- return to work prospects: rationale and clear clinical evidence re:
 - ▶ timeframe
 - ▶ return to work process
 - ▶ reasonable adjustments

Scanning medical documentation – 2



- recommendations re: treatment/support/vocational rehabilitation
- further optional questions:
 - ▶ who provided the service?
 - ▶ are they qualified to formulate diagnoses and formulations
 - ▶ are treatments provided on the basis of clinical needs or on what the provider is able to offer?

B) If assessment/treatment reports are not available

ASK CLINICIANS TO PROVIDE REPORTS
AS THIS IS A PROFESSIONAL OBLIGATION!

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Letter of instruction for psychiatric IME - 1



- focus on case-specific questions
- do not prepare long lists of generic questions do not copy & paste

standard lists of questions **SPEAK DIRECTLY TO ASSESSORS**

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Letter of instruction for psychiatric IME - 2

- general questions re: clinical evidence underpinning - formulation
 - diagnosis
 - prognosis
- case-specific questions re: clinical evidence underpinning -
 - fitness to work and attend insurance/employment-related meetings
 - return to work prospects and timeframe
 - advice on reasonable adjustments
 - disability
 - recommendations re: treatment/support/mediation to:
 - ▶ facilitate return to work
 - ▶ prevent/minimise relapses

In the claims assessor's shoes - 1



What should I write to claimants when requesting a psychiatric IME?

- Containment via personalised, clear, consistent, regular communication
- 'Script'
- Example:

"We are in the process of instructing an independent mental health specialist to carry out a mental health assessment, for the purpose of developing a clearer picture of your psychological difficulties; how these may affect your fitness to work; and which form of professional help you may require to feel better and improve your return to work prospects. You may also consider this as an opportunity to see a mental health specialist for a clinical opinion on your difficulties and appropriate forms of treatment"

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In the claims assessor's shoes - 2



How do I know that treatment is working?

Look in reports for evidence-based shifts in the claimant's:

- perception of self, others, everyday life, workplace, etc.
- way of relating to others
- opening up of their psychological landscape
- new, varied and stable everyday routine
- re-engagement in family and social life
- re-engagement with work environment
 - return to work plans
 - plans-implementation
- ability to consider alternative plans for the future

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In the claims assessor's shoes - 3



What shall I do if ...

- A claimant refuses to provide consent to the IME
- A claimant withdraws their consent to the release to insurers of the IME report

What shall I do if ...

- A claimant refuses to follow clinical recommendations
- Claimant tries to drive the claim

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In the clinician's shoes - 1



- On what basis you may state that a claimant is not able to return to work, or in some cases may permanently be unable to return to work?
- Do you have examples of situations where claimants suffer from mental health conditions but remain fit to work?

In the clinician's shoes - 2



- What do you do when a claimant does not accept your clinical opinion?
- What should happen when you recommend a form of treatment that is not realistically available on the NHS?

Thank you!

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